and rely on a finely tuned piece of programming which would be able to distinguish between particular people's susceptibility to the virus. For instance, a frontline healthcare worker would be more likely to transmit the virus than an asymptomatic child.[7] Programmes used for other infectious diseases would not work; since SARS, influenza and MERS were mostly symptomatic transmissions, models developed for those outbreaks would not be able to provide precise results. In the case of COVID-19, there were simply too many fundamental unknowns to accurately predict the course of the contagion. Most large countries in the West were making the rookie mistake of using severely flawed models by over-relying on the ones already used for known epidemics like influenza or SARS.[8] Hence, it was prudent for Dr Paul and Prof. VijayRaghavan to also think about the extent to which outputs from models would matter in the context of policy relevance in India's response. Emerging evidence was showing that the 'rate of infection'—passing the virus on to another person—was no longer four to six days but had accelerated to two to three days. This had a direct effect on the doubling rate—the number of days in which the number of infected would double.[9]

The two scientists informed the principal secretary of the various scenarios; the greyest of them being that if immediate sufficient mitigation measures were not undertaken, the peak of the COVID-19 wave was four–six weeks away where we would see a total of 15–25 crore people infected and about 12.5 lakh fatalities. Of these, 18 crore infections would likely occur at the peak in early June and almost 20 per cent of those would have moderate to severe disease and require some kind of medical supervision in a hospital due to lack of proven treatment and about 10 per cent would require oxygen support.

India would have to immediately work on aggressive strategies to what the experts were calling 'flatten the curve'. This would mean taking measures to artificially push the intensity of the infection's growth by taking immediate measures. This would buy us time for capacity building, stocking of essentials, disseminating information to the last mile as well as preparing healthcare infrastructure in order to handle the cases. If we would fail to sufficiently flatten the curve, a tsunami of cases would hit us, out of which lakhs would require hospital beds, ICU beds, ventilators and doctors in numbers which India simply did not have readily available at that point in time. Our entire healthcare infrastructure would quickly crumble. As per